



Civil commitment capacity report

State of Utah
Utah Substance Use and Mental Health Advisory Committee
Office of Substance Use and Mental Health
October 2024

To: Judiciary Interim Committee
From: Brent Kelsey, Director, DHHS/SUMH
Subject: Civil commitment commitment capacity report

Purpose

As required by [26B-5-302.5](#), the Department of Health and Human Services (DHHS)/Office of Substance Use and Mental Health (SUMH) submits the following report regarding civil commitment and the Utah State Hospital (USH). State statute outlines the following requirements for the report:

- (1) (a) *The Utah Substance Use and Mental Health Advisory Committee shall study and make recommendations concerning the need for expanded civil commitment capacity in the state, including an analysis of the anticipated impact that any changes to civil commitment standards made during the 2024 General Session will have on the number of individuals subject to civil commitment.*
- (b) *The study and recommendations described in Subsection (1)(a) shall also address the role of the Utah State Hospital in serving patients who are subject to court-ordered treatment, including civil commitment.*
- (c) *The study and recommendations described in Subsection (1)(a) shall also address any additional resources or services needed to decrease the likelihood that individuals who are subject to court-ordered treatment, including civil commitment, will enter or reenter the Utah State Hospital or another inpatient facility.*
- (2) *The Utah Substance Use and Mental Health Advisory Committee shall provide a report on the study and recommendations described in Subsection (1) to the Judiciary Interim Committee at or before the committee's October 2024 interim meeting.*

Executive summary

The DHHS/SUMH analysis of civil commitment capacity in Utah found that the number of

individuals on civil commitment has increased by 41.6% since July 2017 (from approximately 950 to 1,390 individuals), demonstrating a potential need for expanded civil commitment capacity in future years.

The 2024 General Session legislation made several changes to civil commitment standards (described below); a majority of these amendments are procedural, rather than a change to the civil commitment criteria. There is one exception: [HB 203 Involuntary Commitment Amendments, 2024 General Session](#) included a provision that adds individuals with criminal charges that meet civil commitment criteria. This change may increase the overall number of individuals on a civil commitment court order; however, as more individuals are stabilized, civil commitments may decrease over time. When appropriate individuals are ordered to civil commitment, they have increased opportunities to receive prevention and treatment services to reach stability.

It is difficult to estimate how these factors will impact the number of individuals on civil commitment over time. This provision took effect on May 1, 2024, and USH reports there has not been a substantial variation in the total number of individuals on civil commitment.

Utah Code Section [26B-5-306](#) describes the role of the Utah State Hospital: *“The objectives of the state hospital and other mental health facilities shall be to care for all persons within this state who are subject to the provisions of this chapter; and to furnish them with the proper attendance, medical treatment, seclusion, rest, restraint, amusement, occupation, and support that is conducive to their physical and mental well-being”*. This includes adults and youth on civil commitment who are unable to manage at a lower level of care, individuals who have been adjudicated as guilty with a mental condition, individuals who are not guilty by reason of insanity, those who are not competent to stand trial and undergoing restoration, individuals who are awaiting an examination under Title 77, and those with serious mental illness in the custody of the Department of Corrections who require stabilization at USH.

DHHS/SUMH has identified the following resources and services to decrease the likelihood of entry or return to USH or other inpatient facilities. The [Utah Behavioral Health Assessment & Master Plan](#) recommended many of these resources and services to support the Master Plan’s strategic priorities:

- (1) High-acuity community programming
- (2) Diversion programming from inpatient care
- (3) Diversion programming from incarceration
- (4) Programming to stabilize clients during transitions

DHHS/SUMH has recently contracted with the Kem C. Gardner Policy Institute to complete the required report for the Judiciary Interim Committee. DHHS/SUMH will provide this report upon completion.

Potential Legislative Action/Follow-Up

The data on civil commitment court orders from 2017 to 2024 demonstrates an increasing

number of court orders and implies needed increases in capacity for individuals on civil commitment. This supports the 2024 Forensic Mental Health Coordinating Council Report:

“The most immediate need is to consider a proposal for a 30-to-60-bed low acuity treatment unit as quickly as possible. Approvals for funding and building this unit could take 3-5 years, which aligns with the projected need for a 30-bed treatment unit in 2027”.

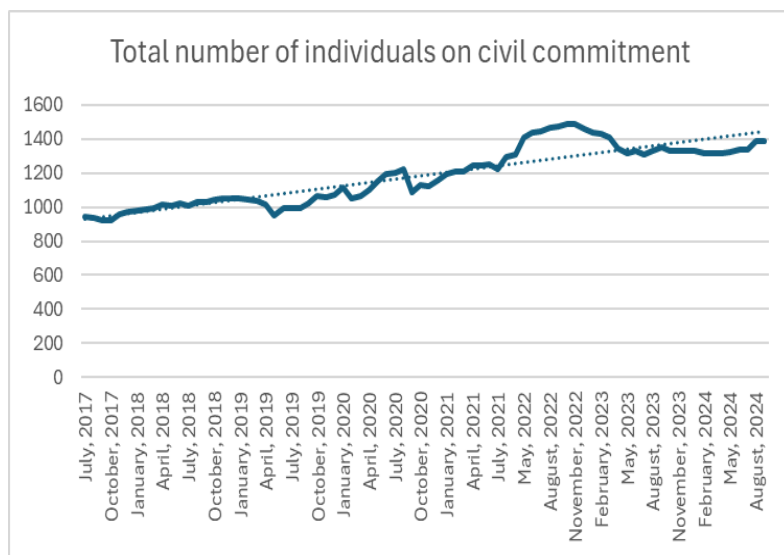
DHHS/SUMH also recommends that the Legislature require courts to identify and implement a process for monitoring how many individuals on civil commitment are committed through the new criteria that includes patients who have been charged with criminal offense. This process will allow the state to track whether civil commitments have increased or decreased because of the new criteria.

Primary report

Need for expanded civil commitment capacity in Utah

DHHS/SUMH has calculated the trend of the total number of individuals on civil commitment in Utah from July 2017 through August 2024. This analysis found that the number of individuals on civil commitment has increased by 41.6% since July 2017, from approximately 950 individuals in July 2017 to just under 1,400 in August 2024 (Table 1). It is important to note that only about 13% of those on civil commitment orders are at USH, with a majority on outpatient civil commitment and maintained in the community.

Table 1. Total number of individuals on civil commitment orders across time



Legislation from the 2024 General Session made changes to civil commitment under [HB 299 Court-ordered Treatment Modifications, 2024 General Session](#), [HB 203 Involuntary Commitment Amendments, 2024 General Session](#), and [HB 94 Civil Commitment Examiner, 2024 General Session](#).

26B-5-331 - Temporary commitment -- Requirements and procedures -- Rights.

a) If a patient was temporarily committed by a peace officer or a mental health

- officer, the amended 2024 code requires that the local mental health authority (or their designee) must notify the peace officer or mental health officer of a patient's release from civil commitment.
- b) Under previous law, patients under temporary civil commitment could be held for a maximum of 24 hours, with a possible extension of 48 additional hours, excluding Saturdays, Sundays, and legal holidays. The amended 2024 code allows patients on civil commitment to be held for a maximum of 72 hours.
 - c) The amended 2024 code changes discharge requirements to require local mental health authorities to provide extensive clinical information to each individual temporarily committed at or before the time the individual is discharged.
- 2) 26B-5-332 Involuntary commitment under court order -- Examination -- Hearing -- Power of court -- Findings required -- Costs.
- a) The amended 2024 code adds a new group of patients who can meet civil commitment criteria. This new group includes those who have been charged with a criminal offense and found incompetent to proceed as a result of a mental illness.
 - b) The amended 2024 code changes discharge requirements to require local mental health authorities to provide extensive clinical information to each individual temporarily committed at or before the time the individual is discharged.
 - c) The amended 2024 code changes the Designated Examiner criteria for determining civil commitment. The new criteria now include psychiatric mental health nurse practitioners or psychiatric mental health clinical nurse specialists who:
 - i) are nationally certified;
 - ii) are doctorally trained; and
 - iii) have at least two years of inpatient mental health experience, regardless of the license the individual held at the time of that experience.
- 3) 26B-5-351 Assisted outpatient treatment proceedings.
- a) The amended 2024 code rephrases criteria for civil commitment to include "based on recent actions, omissions, or behaviors."
 - b) The amended 2024 code clarifies that a court order for assisted outpatient treatment does not create an independent authority to forcibly medicate a patient.
- 4) 26B-6-607 - Temporary emergency commitment - Services for People with Disabilities
- a) The amended 2024 code changes discharge requirements to require local mental health authorities to provide extensive clinical information to each individual temporarily committed at or before the time the individual is discharged.
- 5) 26B-6-608 - Involuntary commitment - Services for People with Disabilities
- a) The amended 2024 code changes discharge requirements to require local mental health authorities to provide extensive clinical information to each individual temporarily committed at or before the time the individual is discharged.

DHHS/SUMH does not anticipate that most of these changes will impact civil commitment

capacity, with one exception. The addition of individuals with criminal charges as meeting civil commitment criteria may increase the number of individuals on civil commitment, as more individuals may potentially meet civil commitment criteria. However, if the appropriate individuals are ordered to civil commitment more quickly, they will have increased opportunities to receive prevention and treatment services. This may decrease civil commitments over time as more individuals are stabilized. It is difficult to estimate how this will impact the number of individuals on civil commitment over time. Since this provision took effect on May 1, 2024, DHHS/SUMH has not seen a substantial variation in the ongoing increasing trendline of the total number of individuals on civil commitment.

Role of the Utah State Hospital in serving patients who are subject to court-ordered treatment

Utah Code Section [26B-5-306](#) describes the objectives of USH and other facilities.

- (1) The objectives of the state hospital and other mental health facilities shall be to care for all persons within this state who are subject to the provisions of this chapter; and to furnish them with the proper attendance, medical treatment, seclusion, rest, restraint, amusement, occupation, and support that is conducive to their physical and mental well-being.*
- (2) Only the following persons may be admitted to the state hospital:*
 - (a) persons 18 years old and older who meet the criteria necessary for commitment under this part and who have severe mental disorders for whom no appropriate, less restrictive treatment alternative is available;*
 - (b) persons under 18 years old who meet the criteria necessary for commitment under Part 4, Commitment of Persons under Age 18, and for whom no less restrictive alternative is available;*
 - (c) persons adjudicated and found to be guilty with a mental condition under Title 77, Chapter 16a, Commitment and Treatment of Individuals with a Mental Condition;*
 - (d) persons adjudicated and found to be not guilty by reason of insanity who are under a subsequent commitment order because they have a mental illness and are a danger to themselves or others, under Section 77-16a-302;*
 - (e) persons found incompetent to proceed under Section 77-15-6;*
 - (f) persons who require an examination under Title 77, Utah Code of Criminal Procedure; and*
 - (g) persons in the custody of the Department of Corrections, admitted in accordance with Section 26B-5-372, giving priority to those persons with severe mental disorders.*

The statute indicates that USH shall care for patients who are subject to court-ordered treatment that cannot be managed in the community, including stabilization and restoration of competency.

Of note, a unique aspect of serious mental illness and court-ordered care involves incarcerated individuals who decompensate, requiring a higher level of psychiatric oversight. In some cases, those individuals may require hospitalization at USH for stabilization (see (2)(g) above). The study will include a review of those individuals and the timing and circumstances leading to decompensation.

Additional resources or services needed to decrease the likelihood that individuals who

are subject to court-ordered treatment will enter or reenter USH or another inpatient facility

DHHS has identified the following resources and services to decrease the likelihood of entry or return to USH or other inpatient facilities. The [Utah Behavioral Health Assessment & Master Plan](#) recommended many of these resources and services to support the Master Plan's strategic priorities:

- 1) High-acuity community programming
 - (a) [Assertive Community Treatment \(ACT\) teams](#): ACT is an evidence-based model that provides community-based services to individuals with serious mental illness "who are at particular risk for hospitalization, homelessness, criminal legal system involvement, and psychiatric crises."
 - (b) Expanded workforce, including paraprofessionals: The Utah Behavioral Health Assessment and Master Plan supports strategies that will "grow and develop a sustainable behavioral workforce across provider types", including paraprofessionals, to improve access to high-quality behavioral health care.
 - (c) Housing: Expanding affordable housing, permanent supportive housing, and housing support services will improve well-being and access to care for people with a serious mental illness. These resources and services can reduce stress and instability for people with a mental illness, improve care transitions for people discharging from hospital stays, and decrease the number of people who are unhoused, among many other benefits.
- 2) Diversion programming from inpatient care
 - a) [Mobile Crisis Outreach Teams](#): These mobile teams of medical and mental health professionals that provide mental health crisis services and coordinate with local law enforcement, emergency medical service personnel, and other appropriate state or local resources play a key role in diversion from higher levels of care.
 - b) [Receiving centers](#): These centers are 23-hour non secure programs or facilities that are responsible for, and provide mental health crisis services to, an individual experiencing a mental health crisis provide stabilization in lieu of accessing emergency rooms.
 - c) Crisis Intervention Teams (CIT): CIT is a community model of partnership between law enforcement, behavioral health professionals, people with lived experience, their families, and other stakeholders. Law enforcement officers can receive training in Crisis Intervention Teams, learning how to identify and appropriately manage a behavioral health crisis.
- 3) Diversion programming from incarceration
 - a) Specialty courts: Individuals can receive referrals to specialty courts such as Drug Court, Family Court, Veterans Court, and Mental Health Court. These courts implement best practices to help clients complete and graduate from their programs. These courts provide oversight and encourage treatment compliance for maintenance and stability in the community.
- 4) Programming to stabilize clients during transitions
 - a) Medicaid justice [waiver](#): On July 2, 2024, Utah Medicaid received approval

through a CMS 1115 demonstration waiver providing limited coverage for a targeted set of services to inmates in a state prison, county jail, or juvenile justice facility for up to 90 days prior to the individual's release date. This new program is designed to help inmates successfully re-enter into their community by providing pre-release Medicaid coverage, service receipt, and quality care, as well as the proactive identification and treatment of both physical and behavioral healthcare needs.

- b) Forensic peer support: These are peer support specialists with experience in both the behavioral health and justice system. Justice-involved individuals can work specifically with forensic peer support specialists to support their recovery.

DHHS/SUMH has recently contracted with the Kem C Gardner Research Institute at the University of Utah to study the potential need for expanded civil commitment capacity in Utah, and to assess the role of USH in court-ordered treatment.